2 Human Rights and Psychiatric Coercion: The Legitimisation Problem for (Social) Psychiatry

2.1 Introduction

Under what circumstances is it professionally, legally and ethically legitimate to restrict the rights of people with psychosocial problems and subject them to various forms of coercion? Such restrictions and coercive measures, which are to be understood as human rights violations according to the United Nations Convention on the Rights of Persons with Disabilities (hereinafter: UN CRPD), take place, for example, when those affected are denied the right to live in their own home on the grounds of an illness and its consequences, when they are assigned a sheltered workshop place instead of support in the primary labour market or when they are taken to a psychiatric hospital and treated against their will.

In almost all countries, there is also the instrument of legal representation, i.e. the possibility of people (commonly called "guardians") taking over certain matters for people with severe psychosocial problems. Legal or statutory representation against the will is a massive social exclusion from the legal system, which is usually encouraged or advocated by mental health services or staff.

With respect to the legitimacy of such restrictions, a bitter dispute has recently arisen between representatives of psychiatric care on the one hand and representatives of affected groups as well as legal and political experts on the other hand. In the view of the United Nations, such a restriction should be fundamentally questioned. "(T)he will and preferences of the person concerned…" must always be taken into account in regard to measures relating to human rights standards for people with disabilities. This is stated in Article 12 of the UN CRPD from 2006 [1]. In other words, according to this position, human rights and individual freedom may be restricted only to the extent that this is possible in accordance with the will and preferences of the person affected by the measure. The entire article and this passage have proven to be among the most controversial legal passages in the recent history of psychiatry. The term "disability" stands—in my view not very

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D. Richter, *Human Rights in Psychiatry*,
https://doi.org/10.1007/978-3-031-98635-2_2

happily—in the UN publications for various limitations that people can experience, including people with "psychosocial disabilities", as it says there.

The convention thus clearly goes beyond the standard that usually prevails in medicine. Respecting the person's will and preferences is not the same as acting "in the person's interest" or "for the person's benefit", as is still customary in medicine, for example, when treating unconscious accident victims. In the case of actions in the person's best interest, a medical decision is made on the basis of assumptions without actually inquiring about the person's wishes and preferences. Depending on the situation, this is often not possible in any other way.

During the COVID-19 pandemic, however, we have had to learn in many different ways that fundamental rights and freedoms can and must be restricted for medical reasons. Without quarantine and lockdown measures, there would have been significantly more cases of illness and death. The relevant empirical research on this topic has impressively confirmed this [2]. The restriction of human rights/freedoms/fundamental rights for medical reasons can therefore be legitimised professionally, ethically and legally because they are often not only in the public interest but also, from a medical point of view, in the interest of the people affected by the measures, even if they do not correspond to their will.

This also applies in a similar way to restrictions on personal freedoms imposed by psychiatry. According to conventional psychiatric justification, there are medical reasons for taking measures against a person's will, such as forced hospitalisation, seclusion/isolation or restraint, as well as detention in a forensic hospital following a criminal offence. If the person is mentally ill in the broadest sense, if they are incapable of judgement, if the intervention against the will of the person concerned does more good than harm and if it is in the person's best interests, then it is justifiable [3]. This justification is based on the four main principles of ethics established in medicine, namely, the principle of *beneficence*, the principle of *non-maleficence*, respect for the *autonomy* of the person concerned and *justice* [4].

A medical measure against the will of the person concerned may therefore be carried out both in an epidemic situation and in the context of psychiatric treatment for ethical and legal reasons if the following criteria are met:

1. It is a defined disease.
2. The disease must have a relatively serious impact on the person affected, on other people or on the population.
3. Effective therapeutic interventions are available.
4. The measure must be for the overall benefit of the person and must not harm them (e.g. as a side effect).
5. The application must be proportionate.
6. Freedoms must be restricted as little as possible.

This is much like the conventional justification for restricting civil liberties on medical grounds. In recent years, psychiatry, which essentially represents this position in connection with coercion in treatment, has had to address criticism from unexpected quarters. First, the United Nations and its suborganisations, such as the

World Health Organisation (WHO), have repeatedly drawn attention to the aspects of violated human rights, such as those arising from coercive measures. In an additional commentary on Article 12 of the UN CRPD, the responsible UN committee explained that, contrary to the legal traditions of many countries, the article calls for the separation of mental capacity and legal capacity [5]. This means that people who are certified as lacking mental capacity should be allowed to refuse medical treatment and thus also coercive psychiatric measures, as they retain their legal capacity in principle. The instrument of legal representation (e.g. guardianship) would thus become obsolete, at least if it were to happen against the will of the person concerned.

Second, various UN rapporteurs have repeatedly pointed out the violation of the rights of people with psychosocial problems and denounced coercive measures such as restraint, seclusion/isolation and forced medication. The controversial demand in psychiatric circles is that coercive measures should be abolished as a matter of principle. The disputes between the psychiatric associations and the suborganisations of the United Nations reached a temporary climax when the UN Human Rights Council appointed a rapporteur to deal with torture, abuse and degrading treatment in medical institutions [6]. Psychiatric institutions were particularly affected by this report, as it called for the revision of legal regulations that allow coercive measures in general: "Legislation authorizing the institutionalization of persons with disabilities on the grounds of their disability without their free and informed consent must be abolished". In addition, the main report established a particular link between coercion and the conventional biomedical paradigm in psychiatry. Accordingly, the report calls for a fundamental change: "Coercion, medicalization and exclusion, which are vestiges of traditional psychiatric care relationships, must be replaced with a modern understanding of recovery and evidence-based services that restore dignity and return rights holders to their families and communities" [7: 18].

Third, the World Health Organisation (WHO) recently published new guidelines for psychiatric care that focus on the avoidance of coercion and call for human rights to be respected in psychiatry. Here, too, a departure from the biomedical and primarily medication-based paradigm is called for. The guidelines explicitly expect WHO member states to ensure the right of affected people to refuse any form of psychiatric intervention [8].

The circumstances described by the UN organisations were not compatible with the traditional medico-psychiatric self-image. Furthermore, different ethical judgements regarding the importance of human rights can also be assumed. While the UN CRPD and various other UN documents ascribe a prominent position to human rights and civil liberties, this is not the case according to the current standard work on biomedical ethics. It states that "…human rights are no more fundamental than moral virtues in a universal morality…" [4, p. 4]. In other words, human rights are no more important than, for example, professional medical ethics.

For these reasons, there have been few relevant changes in the core area of mental health care to date. At best, there have been legal changes in some countries in the nonmedical area of support for people with disabilities, which can certainly be seen as being in line with the UN CRPD. The extent to which this has actually

improved the situation of the people concerned remains to be seen in future evaluations.

However, it was not only conventional biomedically orientated psychiatry that had problems with this criticism and the associated demand for the abolition of measures against the will of the person concerned. One might have expected social psychiatry to adopt these demands and initiatives. Although this is happening to a limited extent, the reactions of many professionals are different—among nurses, too [9]. The general demand for the abolition of coercion in psychiatry is highly controversial even within social psychiatry, as I will show in detail in the following section.

The conflict mentioned at the beginning between those who want to abolish coercion and those who want to retain coercion as a last but indispensable means can, in my opinion, be resolved, as I hope to be able to explain in the course of the book. To this point, we have basically only been talking about the abolition of measures against a person's will in the context of care and support. However, I am convinced that the UN CRPD and related initiatives such as "QualityRights" [10] can also be read as a guide for the implementation of contemporary psychiatry in the sense of psychosocial support. In the following section, the various topics addressed by the Convention are presented in more detail: which human rights violations are meant, and what would be the consequences of consistent implementation?

The first important thing to understand about the Convention on the Rights of Persons with Disabilities is that it does not apply only to people with mental or psychosocial problems. The Convention applies to people with any form of disability, i.e. both physical and other problems. The central requirement of the Convention is the nondiscrimination of people with disabilities. People with disabilities should be treated in the same way as people without disabilities. They should enjoy the same rights as all other people, and this also applies to their rights in the healthcare system.

Later in the book, as indicated earlier, I will discuss the terminology used here, in particular why the terminology of "disability" is not useful in my view and why a neutral and inclusive term such as "psychosocial problems" seems more appropriate. In the following section, however, I follow the official terminology of the Convention to avoid misunderstandings.

2.2 Human Rights and the Social Model of Disability

Article 1 of the UN CRPD describes how disabilities should be understood, namely, "interaction with various barriers [that] may hinder their full and effective participation in society on an equal basis with others". This means that the concept of disability follows the so-called social model [11], in which the problem no longer lies primarily in the person, as was the case in earlier medical models. Previously, the person had to adapt to an environment of some kind, for example, by choosing the living situation according to the person's possibilities and skills. As a rule, no further development then took place. On the other hand, concepts such as "residential coaching" provide support services that help people cope with housing situations

that were previously considered impossible for them. In other words, the problem of disability arises in interaction with the social environment, and the environment must be adapted to the person in such a way that the person can ideally live according to their preferences.

Figure 2.1 summarises the key features and implications of both the medical model and the social model of disability (see also [12]). As already indicated, the medical model of disability assumes that disability is a characteristic of the person. The person *is* disabled. Disabled people can integrate by adapting to their environmental conditions. If this is not possible, they are supported in special institutions such as homes or workshops. In contrast, the social model of disability is based on the decisive reasons in the person's environment. A person becomes disabled because the social environment does not respond flexibly enough to the person's needs. In contrast to social integration, inclusion takes place here by adapting the social environment to the person.

Behind the entire Convention is the aspiration to "enable persons with disabilities to live independently and participate fully in all aspects of life", as Article 9 states. This means the full inclusion of people in society and the opportunity to achieve everything that people without disabilities can achieve.

Consequently, full participation also means legal equality. The already mentioned, highly controversial Article 12 demands that people with disabilities retain their legal capacity and, if necessary, receive support to exercise their rights. This article is generally interpreted to mean that legal representation against the person's will is no longer permitted. These are to be replaced by supported decision-making, which should essentially consider the person's will and preferences.

Article 15, sentence 1 of the UN CRPD reads: "No one shall be subjected to torture or to cruel, inhuman or degrading treatment or punishment. In particular, no one shall be subjected without his or her free consent to medical or scientific experimentation". This sentence defines involuntary detention and treatment as close to or—as in various UN publications—as torture and degradation. It follows that any

Medical model	**Social model**
The reason for the disability lies in the person → The person is disabled	The reason for the disability lies in the social environment, which does not react flexibly enough to the needs of the person concerned → The person becomes disabled
Adaptation of the person concerned to the environment or care in a special institution → Integration	Primary adaptation of the environment to the needs of the person concerned; no care in a special institution → Inclusion

Fig. 2.1 Medical and social model of disability

involuntary form of medical intervention in connection with disability must be avoided. Treatment in psychiatric settings must therefore be exclusively voluntary. While a negative formulation is used here, Article 25, which describes access to healthcare services, explicitly states that states undertake "…to provide care of the same quality to persons with disabilities as to others, including on the basis of free and informed consent…".

The independent living of persons with disabilities is set out in Article 19. It states, among other things, that "(p)ersons with disabilities have the opportunity to choose their place of residence and where and with whom they live on an equal basis with others and are not obliged to live in a particular living arrangement (…)". They shall have access to support services, "including personal assistance necessary to support living and inclusion in the community, and to prevent isolation or segregation from the community". This is intended to emphasise that people with disabilities should no longer have their living arrangements dictated to them but should live as they wish and receive the necessary assistance. Placements against their will in care homes or similar settings would therefore no longer be permitted. In this context, Article 25 calls for nationwide outpatient care, including in rural areas, with regard to healthcare services.

According to the Convention, independent living and full participation also includes equal access to educational opportunities. Article 24 regulates inclusive education, which is intended to prevent the segregation of programmes for people with and without disabilities. Building on the education article, Article 27 formulates the right to "the opportunity to gain a living by work freely chosen or accepted in a labour market and work environment that is open, inclusive and accessible to persons with disabilities", "including equal opportunities and equal remuneration for work of equal value". Employment in sheltered workshops is therefore not equivalent, and people with disabilities must first be given access to the general labour market if they want to. Finally, Article 28 formulates the right to an adequate standard of living "for themselves and their families, including adequate food, clothing and housing, and to the continuous improvement of living conditions".

If we summarise the articles of the Convention just mentioned, which are particularly relevant for people with psychosocial problems, we can derive clear mandates for psychiatry in general and for social psychiatry in particular. People with psychosocial problems should live, work and be supported in the way they wish. Mental health support in this sense would mean [1] exclusively voluntary treatment, [2] supported forms of living according to the person's preference, [3] ensuring education and vocational training, [4] ensuring access to the general labour market, [5] preventing poverty, and [6] new forms of decision-making that prioritise the person's will and preferences.

In principle, most professionals in psychiatric or psychosocial care are in agreement with the above-mentioned mandates. However, I would like to use two current controversies to show that this agreement has clear limits and that, when push comes to shove, the principles no longer apply. The first is the issue of coercion and therapy, which has already been mentioned, and the second is assisted suicide or

medical aid in dying for people with psychosocial problems, i.e. the coercion to live that prevails in most countries of the world.

2.3 The Controversy Surrounding the Use of Coercion in Social Psychiatry

As mentioned above, the use of coercion in mental health care is highly controversial. The controversy continues to take place in various areas of psychiatry, unfortunately only rarely in mental health nursing. With a few exceptions [13, 14], aspects such as the UN CRPD and its implications are rarely discussed. For this reason, the discussion in social psychiatry is traced here.

Recently, the controversy surrounding coercion in treatment has been discussed in various German- and English-language psychiatry journals. On the one hand, they were leading figures from the social psychiatry community; on the other hand, they were equally well-known professionals as well as representatives of those affected. The subject of the controversy was the question of whether psychiatric care can fundamentally do without coercion and what consequences such a renunciation would entail.

This topic and the associated controversies are not new. In the early 1960s, the US psychiatrist and psychoanalyst Thomas Szasz concluded, "There is no medical, moral or legal justification for involuntary psychiatric interventions. These are crimes against humanity" [15: 268]. The reason for this conclusion was as simple as it was controversial: In Szasz's view, the idea of mental illness was a "myth". Mental illness had been "invented" by psychiatry, deviating from the central insights of medicine. In contrast to physical illnesses, which are based on demonstrable changes to the body structure (e.g. a tumour or a fracture), a new criterion was introduced in the case of psychiatry, that is, functional change. According to Szasz, however, functionally explained diseases are not "real" diseases, as they were not discovered but invented.

However, what should happen to people who have committed a serious norm violation, such as a criminal offense, against the background of a supposed mental illness? Here too, Szasz's answer was as simple as it was controversial: These people should be imprisoned. "Institutions of a punitive nature (prisons) should house those whom society wishes to segregate. The primary purpose of this institution should be the establishment of public safety" [16: 170]. People who were considered mentally ill were, in Szasz's opinion, nothing more than Jews or non-white people, who at various points in history were declared scapegoats.

Thomas Szasz, who, together with the British psychiatrist R.D. Laing, the American sociologist Erving Goffman and the French philosopher Michel Foucault, is now counted among the founders of "anti-psychiatry", was not fundamentally opposed to psychiatric support for people who voluntarily sought it. According to his interpretation, these people were concerned primarily with questions and uncertainties about how to lead their lives, such as the realisation of life goals. Martin Zinkler and Sebastian von Peter make a very similar argument in their 2019 article

"End Coercion in Mental Health Services—Toward a System Based on Support Only" [17]. They also propose that law enforcement authorities should no longer be able to take people to a psychiatric hospital against their will, as is currently possible. However, their reasoning is different to that of Szasz. Zinkler and von Peter's reform ideas are based on the aforementioned UN CRPD, which prohibits discrimination against people with physical or mental disabilities compared with people without disabilities. Accordingly, people with and without disabilities must be treated equally in principle. "The principle of non-discrimination in the Convention stipulates that persons with an assumed or diagnosed mental illness must not be treated legally different than persons without this attribution" [17: 4].

These considerations do not neglect to support those who refuse hospital treatment. Alternatives include assistance with social problems such as homelessness and poverty, as well as home treatment, i.e. acute psychiatric treatment in the patient's own home. However, this form of psychiatry, conceived exclusively as supportive, is intended to dispense with the traditional "dual function" of treatment and order/control by discarding the control function. The authors hope that this will lead to greater acceptance of psychiatric support services if the users realise that the possibilities for coercive measures can no longer take place in psychiatry but at best in the area of police and criminal law.

The article by Martin Zinkler and Sebastian von Peter provoked harsh reactions. According to psychiatrists Maximilian Gahr and Manfred Spitzer in a rebuttal [18: 136], the considerations are "…in terms of psychiatric history… a step backwards by approximately 200 years…" (Translations from the German have been provided by the author of the book). According to their assessment, the approach of psychiatry operating exclusively without coercion is essentially "…anti-patient, because it can run counter to a person's interest in receiving professional help when they are ill". [18: 137] Psychiatrist Peter Brieger and psychologist Susanne Menzel even characterised the approach as "inhumane" [19: 297]. The principle of exclusively non-coercive psychiatric care inadmissibly emphasises the aspect of self-determination over the principles of care and non-harm.

The psychiatrist Tilman Steinert argued somewhat differently against the ideas of Martin Zinkler and Sebastian von Peter. His central objection relates to the difference between physical and mental illnesses. This is to be seen in the fact "…that mental illnesses affect the organ responsible for a person's volitional decisions, the brain. All mental illnesses can—in a relatively broad sense—be understood as functional disorders of the brain" [20: 30]. Due to this difference, mental illnesses have a special status, as they can restrict the ability to make free-will decisions. Against this background, it is justifiable in principle that medical professionals should be allowed to order measures that restrict freedom in the interests of other people, analogous to infection control measures that can make quarantine mandatory for people with existing mental capacity.

However, the argument that professionals should be allowed to order measures in the interests of people with "mental illnesses" was then drastically contradicted in a statement by two German associations of service users. The view associated with the described approach of psychiatry without coercion reflects "… what those with

experience of psychiatry have long criticised about the existing psychiatric system. Those affected regularly draw attention to the fact that psychiatric violence runs counter to the medical principle of non-harm and is considered unethical and inhumane" [21: 36]. The associations objected to what they saw as the instrumentalisation of the benefit of those affected by the critical positions cited above in relation to the concept of psychiatry operating exclusively without coercion. This instrumentalisation was part of the "psychiatric violence" experienced by those affected.

In summary, the perspectives on autonomy, freedom of choice and "mental illness" differ fundamentally. On the one hand, there is the conventional psychiatric and social psychiatry view that an illness impairs the exercise of one's own rights and therefore requires a proxy decision by medical professionals under certain circumstances. On the other hand, those affected dispute that anyone should be allowed to act as a proxy without further ado, regardless of whether the impaired person is considered "mentally ill".

Incidentally, the latter position also corresponds to the views of the United Nations CRPD. Accordingly, mental disabilities must not lead to legal restrictions. This means that substitute decision-making, for example, by a guardian or a legal representative, is basically not permitted. This legal position has triggered considerable debate [22, 23] but also considerable resistance [24] in the psychiatric community.

2.4 The Controversy Surrounding Medical Aid in Dying for People with Psychosocial Problems

The second topic also revolves around the question of the autonomy of people with mental health problems. Apart from the direct issue of coercion already mentioned, no other discussion is likely as emotional as the discussion concerning assisted suicide or medical aid in dying (MAID) for people with psychosocial problems. In various European countries, MAID is also permitted for people with mental health problems, for example, in Switzerland, Belgium and the Netherlands.

However, to be clear from the outset, a ban on MAID is a variant of the administrative or social coercion described in the introduction. People are forced either to continue living with their suffering or to seek an undignified way out, or even one that harms other people. Of course, many readers will not necessarily take note of this section with approval, as MAID has ethical, medical-psychiatric or even religious connotations. I am not primarily concerned here with the question of the permissibility of MAID but with the use of psychiatrically legitimised coercion against people who want to end their lives.

I would therefore like to make two brief comments in advance. For the sake of transparency, it must be reported here that I am involved in this controversy in German-speaking countries and that I am a supporter of MAID for people with psychosocial problems [25, 26]. In the course of a thorough examination of this issue, and this is the second remark, I have drawn terminological consequences for myself that clearly distinguish "suicide" from "MAID". According to the American

Association of Suicidology [27] and German bioethicists [28], suicide is an act in the context of an acute mental disorder, whereas MAID is a well-considered step that does not occur in the context of an acute problem.

From the point of view of many psychiatric professionals, there are various reasons against assisted suicide. First, there is the history of psychiatry, which, with the "euthanasia" of people with mental impairments during the reign of National Socialism, contains a dark chapter that cannot be surpassed. The conceptual history of social psychiatry is—which may surprise some readers—overshadowed by the ideas of racial hygiene [29]. It is therefore all the more understandable that postwar social psychiatry, particularly in Germany, sought to draw a clear dividing line between mental health care and the killing of impaired people. Against this background, Asmus Finzen, one of the best-known reform psychiatrists in Germany and Switzerland, has lamented the fact that Belgium and the Netherlands, for example, have even considered authorising active euthanasia for people with mental illness [30].

Another reason is the medical categorisation of suicidality as a pathological phenomenon. What many psychiatric clinicians today basically take for granted, namely, that suicide predominantly has a psychopathological background, has not always been the case and is still not the case in parts of the world today. For a long time, suicide was a moral, religious and even illegal act that has only recently been medicalised and pathologised [31]. It is well known that one of the central tasks of psychiatric care is to prevent suicide; in almost all legal-psychiatric contexts, suicidal acts are a reason to use coercion against the person concerned and, under certain circumstances, to treat them against their will. This distinguishes psychiatry from other medical disciplines. If a person with severe diabetes mellitus endangers themselves by not adhering to their diet, this usually has no legal consequences.

In addition, according to another explicitly social psychiatry argument against assisted suicide, the possibility of assisted suicide would send out a "devastating signal" if it were to bring mental illness close to incurability, according to sociologist Silvia Krumm, in a pro and con discussion with me [32]. This would destroy any hope of recovery and would result in a life with the illness, which would run counter to the recovery concept promoted in social psychiatry.

From the conventional psychiatric perspective and the social psychiatry perspective, no distinction is usually made between suicide and MAID. A MAID requires a well-considered decision, which is often not the case with suicide. The pathologisation of an act that seeks to bring about one's own death now leads to a circular argument, as suicidal behaviour can be a symptom of a mental disorder. Anyone who wants to end their life is suicidal, and anyone who is suicidal is mentally ill and can therefore lack capacity, according to the abbreviated argument. When clinicians have to assess capacity, as is often the case in the run-up to MAID, "… determining competence is difficult when the wish to die is part of the underlying syndrome itself", according to the influential psychiatrist Paul Appelbaum, who is highly critical of assisted suicide [33: 316].

Once again, it is therefore a question of the autonomous decision of people who have psychosocial problems. If life with these psychosocial problems is

experienced as no longer bearable, then both conventional psychiatry and many people in the social psychiatry community deny those affected the right to make an autonomous decision about this step. When the Swiss Academy of Medical Sciences published new guidelines on assisted suicide in healthcare a few years ago, which also included people with mental health problems, large sections of the medical community reacted with drastic rejection. "Any objectivity and any shared subjectivity with regard to the question of whether suffering is unbearable are thus sacrificed on the altar of the principle of autonomy" [34: 911]" When the highest court in Germany, the Federal Constitutional Court, called for new legal principles in dealing with assisted suicide, many clinicians objected because they saw this as a kind of glorification of suicide as a freedom right.

From the perspective of rights for people with disabilities, as set out in the UN CRPD, for example, the exclusion of mental health problems as a cause of suicide is a fundamental form of discrimination. People with physical illnesses are granted MAID, whereas this is not the case for people with profound psychosocial problems. This clearly contradicts the nondiscrimination principle of the UN CRPD. If people with unbearable psychosocial conditions are denied this step as part of a well-considered conclusion, in my opinion, this is not compatible with a recovery perspective of nursing and mental health care. For me, the contradiction is obvious: we promote recovery and empowerment so that people with psychosocial problems can regain control of their lives and the decisions associated with them. Recovery also means taking risks and not ruling out everything that could lead to problematic situations from the outset. This aspect of the recovery process is referred to as "positive risk-taking" [35]. We also want to move away from a paternalistic attitude in social psychiatry and mental health nursing, which suggests that it knows what is good for the person concerned. Ultimately, however, this means that we have to allow the person concerned to make decisions that we do not necessarily like. In the particular case of MAID, this includes the decision not to continue living. It is not acceptable to say take your affairs back into your own hands but only for as long as it suits me.

Furthermore, the illness concept of suicide should not be left as the only possible interpretation. It is possible to commit suicide for rational reasons [36]. Recently, there has been increasing criticism of so-called psychological autopsy studies, which retrospectively search for psychological causes of suicide and find these causes in 70–90 percent of cases. From a methodological point of view, such retrospective studies are highly dubious [37]; interestingly, scientifically adequate studies that are conducted prospectively have not yet found any stable predictors of suicide [38].

From an epidemiological perspective, doubts about an exclusively pathologising perspective are also justified. Social developments, such as the financial crisis at the end of the 2000s, have led to significantly increased suicide rates in Greece [39] and Spain [40]. In German history, the reunification of the two German states, East Germany and West Germany, is, to a certain extent, a natural experiment that suggests an inverse relationship [41]. The suicide rates in the territory of the former GDR were already significantly higher than those in the West before the Second

World War and remained high during the socialist regime. However, the high suicide rates were kept secret and not reported to international organisations such as the WHO, as this did not fit with the image of a functioning socialism. With reunification after 1990, suicide rates in the former GDR fell dramatically, and in recent years, they have levelled off with decreasing rates in the West. Given the socioeconomic crisis years after reunification and the differences between East and West that still persist today, it cannot be assumed that the psychosocial burden on people in the former GDR has also fallen significantly. This means that suicides are also subject to completely different influences than primarily psychopathological characteristics. The reduction in suicide rates in East Germany is likely not due to a dramatic improvement in the mental health of the people living there.

On a completely different level, the pathologising view of suicidality is also experienced as an epistemic injustice in the discourse on disabilities. The concept of epistemic injustice, which is explained in more detail elsewhere and which has recently been increasingly discussed in psychiatry [42], describes the situation in which any other perspectives of the persons affected are not taken into account and are not heard in the scientific, political or legal realm. Analogous to racism, this discourse describes discrimination by non-disabled people (*ableism*) and healthy people (*sanism*) but also by people who are not suicidal [43]. Many people with suicidal experiences reject a pathologising view of this phenomenon as suicidism.

Finally, for the sake of completeness, it should also be mentioned that MAID is highly controversial in the disability movement. There are repeated fears that MAID is used as a means of pressurising people with disabilities to end their life because it places a burden on the family and causes costs. Empirically, however, this fear has little basis. Although there are no relevant studies related to psychosocial problems, data related to physical illness suggest that MAID is sought primarily by people with high social status and against a background of pronounced individuality and a lack of religious commitment [44, 45]. These studies suggest that social pressure does not appear to play a significant role.

2.5 Conclusion: The Decision-Making Dilemma of (Social) Psychiatry

How much room for manoeuvring do we give people who we diagnose as "severely mentally ill"? And is it justified to treat people with "severe mental illness" differently from people with physical illnesses, who may also have problems making illness-related decisions?

These questions lead psychiatry, but above all social psychiatry and mental health nursing, into a dilemma. On the one hand, people with severe psychosocial problems should be able to make decisions about their own affairs as independently and autonomously as possible. On the other hand, it is assumed that "mental disorder" can lead to decisions that may not be in the person's best interests. This is aggravated by the fact that political and legal positions, as well as associations of those affected, categorise decisions against a person's will as a violation of human

rights according to the UN CRPD. Essentially, the question is what is considered pathological and to what extent "illness" or "disorder" can be used to legitimise interference with human rights.

The fact that this is not unproblematic is repeatedly emphasised in the relevant literature. It is not necessary to return to the positions of Thomas Szasz from the 1960s. The British psychiatrist George Szmukler recently noted the legitimisation problems of coercion in psychiatry on various occasions and suggested already in 2006 that these problems could be solved by legal means. His solution is that a common law (*fusion law*) is needed that does not discriminate between mental and non-mental illnesses but only takes mental capacity into account [46, 47]. Ultimately, however, his argument—which, in my view, is entirely understandable—has not been able to prevail, especially in psychiatry. The following chapter addresses in detail the reasons cited in psychiatry for legitimising measures against a person's will.

References

1. United Nations. UN convention on the rights of persons with disabilities. 2006. Available from: https://www.un.org/development/desa/disabilities/convention-on-the-rights-of-persons-with-disabilities/convention-on-the-rights-of-persons-with-disabilities-2.html.
2. Yakusheva O, van den Broek-Altenburg E, Brekke G, Atherly A. Lives saved and lost in the first six month of the US COVID-19 pandemic: a retrospective cost-benefit analysis. PLoS One. 2022;17(1):e0261759.
3. Ryan CJ, Bartels J. Involuntary hospitalization. In: Bloch S, Green SA, editors. Psychiatric ethics. 5th ed. Oxford: Oxford UP; 2021. p. 279–99.
4. Beauchamp TL, Childress JF. Principles of biomedical ethics. 8th ed. Oxford: Oxford UP; 2019.
5. Snellgrove BJ, Steinert T. Einwilligungsfähigkeit vor dem Hintergrund der UN-Behindertenrechtskonvention. Forens Psychiatr Psychol Kriminologie. 2017;11(3): 234–43.
6. UN General Assembly. Report of the special rapporteur on torture and other cruel, inhuman or degrading treatment or punishment, Juan E Méndez. 2013. Contract No.: A/HRC/22/53.
7. UN General Assembly. Report of the Special Rapporteur on the right of everyone to the enjoyment of the highest attainable standard of physical and mental health. 2017. Contract No.: A/HRC/35/21.
8. WHO. Guidance on community mental health services: promoting person-centred and rights-based approaches. Geneva: World Health Organisation; 2021.
9. Galbert I, Azab AN, Kaplan Z, Nusbaum L. Staff attitudes and perceptions towards the use of coercive measures in psychiatric patients. Int J Ment Health Nurs. 2023;32(1):106–16.
10. Funk M, Bold ND. WHO'S QualityRights initiative: transforming services and promoting rights in mental health. Health Hum Rights. 2020;22(1):69–75.
11. Shakespeare T. The social model of disability. In: Davis LJ, editor. The disability studies reader. 2. London: Routledge; 2006. p. 197–204.
12. Reynolds JM. Theories of disability. In: Reynolds JM, Wieseler C, editors. The disability bioethics reader. New York: Routledge; 2022. p. 30–8.
13. Fitzpatrick JJ. Moral and ethical issues in mental health. Arch Psychiatr Nurs. 2016;30(6):647.
14. Barros S, Rodrigues J, Alves TC, Almeida AB. Nursing and the rights of people in the field of mental health. SciELO Brasil. 2021;75:e75suppl301.
15. Szasz TS. The myth of mental illness: foundations of a theory of personal conduct. New York: Harper; 1974.

16. Szasz TS. Open doors or civil rights for mental patients? J Individ Psychol. 1962;18(2):168–71.
17. Zinkler M, von Peter S. End coercion in mental health services—toward a system based on support only. Laws. 2019;8(3):19.
18. Gahr M, Spitzer M. Probleme einer psychiatrischen Versorgung ohne Zwang: Krankenhaus oder Gefängnis? Recht und Psychiatrie. 2020;38:135–7.
19. Brieger P, Menzel S. Psychiatrie ohne Ordnungsfunktion? – Kontra. Psychiatr Prax. 2020;47:297–8.
20. Die ST, der Psychiatrie D. Recht und. Psychiatrie. 2021;39:28–34.
21. Bundesverband Psychiatrie-Erfahrener, Bundesarbeitsgemeinschaft Psychiatrie-Erfahrener. Nicht in unserem Namen! Zur Kritik am Konzept einer freiwilligen Psychiatrie aus Betroffenenperspektive. Recht und Psychiatrie. 2021;39:35–8.
22. Szmukler G. "Capacity", "best interests", "will and preferences" and the UN convention on the rights of persons with disabilities. World Psychiatry. 2019;18(1):34–41.
23. Scholten M, Gather J. Adverse consequences of article 12 of the UN convention on the rights of persons with disabilities for persons with mental disabilities and an alternative way forward. J Med Ethics. 2018;44(4):226–33.
24. Appelbaum PS. Protecting the rights of persons with disabilities: an international convention and its problems. Psychiatr Serv. 2016;67(4):366–8.
25. Richter D. Assistierter Suizid/assistierte Selbsttötung für Menschen mit schweren psychischen Störungen – Pro. Psychiatr Prax. 2016;43(8):411–2.
26. Richter D. Unerträgliches Leiden und autonome Entscheidung: Warum Menschen mit psychischen Erkrankungen das Recht auf Sterbehilfe nicht verwehrt werden darf. In: Böhning A, editor. Assistierter Suizid für psychisch Erkrankte: Herausforderung für Psychiatrie und Psychotherapie. Bern: Hogrefe; 2021. p. 37–61.
27. AAS. "Suicide" is not the same as "Physician Aid in Dying": American Association of Suicidology. 2017. Available from: http://www.suicidology.org/Portals/14/docs/Press%20Release/AAS%20PAD%20Statement%20Approved%2010.30.17%20ed%2010-30-17.pdf.
28. Gather J, Vollmann J. Suizidprävention und ärztlich assistierte Selbsttötung. Ein unauflösbarer ethischer Widerspruch für Psychiater? Nervenheilkunde. 2015;34:430–4.
29. Schmiedebach HP, Priebe S. Social psychiatry in Germany in the twentieth century: ideas and models. Med Hist. 2004;48(4):449–72.
30. Finzen A. Die neue Euthanasie in Belgien—Wie steht es um die psychisch Kranken? Psychiatr Prax. 2015;42:411–2.
31. Barbagli M. Farewell to the world: a history of Suicide. London: Polity Press; 2015.
32. Krumm S. Assistierter Suizid/assistierte Selbsttötung für Menschen mit schweren psychischen Storungen – Kontra. Psychiatr Prax. 2016;43(8):412–3.
33. Appelbaum PS. Should mental disorders be a basis for physician-assisted death? Psychiatr Serv. 2017;68(4):315–7.
34. Ducor P, Kiefer B. Grundsatz der Autonomie: ein letztes Sakrament? Schweizerische Ärztezeitung. 2018;99:910–2.
35. Morgan S, Andrews N. Positive risk-taking: from rhetoric to reality. J Ment Health Train Educ Pract. 2016;11(2):122–32.
36. Stefan S. Rational Suicide, Irrational Laws. Examining current approaches to Suicide in policy and law. New York: Oxford University Press; 2016.
37. Hjelmeland H, Dieserud G, Dyregrov K, Knizek BL, Leenaars AA. Psychological autopsy studies as diagnostic tools: are they methodologically flawed? Death Stud. 2012;36(7):605–26.
38. Franklin JC, Ribeiro JD, Fox KR, Bentley KH, Kleiman EM, Huang X, et al. Risk factors for suicidal thoughts and behaviors: a meta-analysis of 50 years of research. Psychol Bull. 2017;143(2):187–232.
39. Economou M, Madianos M, Theleritis C, Peppou LE, Stefanis CN. Increased suicidality amid economic crisis in Greece. Lancet. 2011;378(9801):1459.
40. Lopez Bernal JA, Gasparrini A, Artundo CM, McKee M. The effect of the late 2000s financial crisis on suicides in Spain: an interrupted time-series analysis. Eur J Pub Health. 2013;23(5):732–6.

41. von den Driesch E. Unter Verschluss: Eine Geschichte des Suizids in der DDR 1952–1990. Frankfurt M./New York: Campus; 2021.
42. Crichton P, Carel H, Kidd IJ. Epistemic injustice in psychiatry. British J Psychiatry Bullet. 2017;41(2):65–70.
43. Baril A. Suicidism: a new theoretical framework to conceptualize suicide from an anti-oppressive perspective. Disability Stud Quarterly 2020;40(3).
44. Battin MP, van der Heide A, Ganzini L, van der Wal G, Onwuteaka-Philipsen BD. Legal physician-assisted dying in Oregon and The Netherlands: evidence concerning the impact on patients in "vulnerable" groups. J Med Ethics. 2007;33(10):591–7.
45. Steck N, Junker C, Zwahlen M, for the Swiss National Cohort. Increase in assisted suicide in Switzerland: did the socioeconomic predictors change? Results from the Swiss National Cohort. BMJ Open. 2018;8(4):e020992.
46. Dawson J, Szmukler G. Fusion of mental health and incapacity legislation. Br J Psychiatry. 2006;188(6):504–9.
47. Szmukler G. Men in white coats: treatment under coercion. Oxford: Oxford University Press; 2018 01 Dec 2017.